

Pain Assessment and Management

Multimodal Pain Strategies and Opioid Safety

Regulation: 42 CFR §482.13(c)(1) | §482.21 |
Joint Commission NPSC

POLICY OVERVIEW

Effective pain management is both a patient right and a quality and safety priority. In the context of the opioid crisis, hospitals must balance adequate pain control with opioid stewardship. A multimodal pain management approach reduces opioid reliance while improving patient comfort and outcomes.

KEY REQUIREMENTS

1 Pain Assessment

Pain must be assessed using a validated, patient-appropriate tool (NRS 0-10 for cognitively intact adults, FLACC or Wong-Baker for children, CPOT for non-verbal patients). Pain assessment must be performed on admission, with every vital sign set, before and after pain interventions, and at discharge.

2 Multimodal Pain Management

Pain management plans must consider multimodal approaches including: non-pharmacological interventions (positioning, ice/heat, distraction), non-opioid medications (acetaminophen, NSAIDs, gabapentinoids), regional/neuraxial techniques, and opioids reserved for when other methods are insufficient.

3 Opioid Prescribing Safety

Opioid orders must include: specific drug, dose, route, frequency, and indication. PRN opioid frequency must be clinically appropriate. Concurrent CNS depressants (benzodiazepines, muscle relaxants) must be prescribed with caution and documented rationale.

4 High-Risk Patient Monitoring

Patients receiving opioids must be monitored for respiratory depression, sedation, and oxygen saturation. High-risk patients (elderly, opioid-naïve, obese, sleep apnea) require enhanced monitoring. Naloxone must be available at the bedside for high-risk patients.

5 Opioid Use Disorder Considerations

Patients with known or suspected opioid use disorder must receive concurrent addiction medicine consultation or referral. Medication-assisted treatment (MAT) should be continued or initiated during hospitalization. PDMP (Prescription Drug Monitoring Program) must be checked before prescribing opioids.

COMPLIANCE NOTES

Pain reassessment following intervention is required and must be documented. Patient satisfaction with pain management is measured through HCAHPS surveys and affects VBP reimbursement. Opioid prescribing data must be reviewed for outlier practitioners.

COMMON SURVEY CITATIONS

Required under §482.13(c)(1) – Right to appropriate assessment and management of pain, and Joint Commission Pain Management Standards.